

CLINICAL BRIEFING FOR GYM OPERATORS

Appetite drops. Protein needs don't.

Why nutrition is half the job in GLP-1 care.

Intake falls 16% to 39%.

GLP-1 therapy reduces energy intake by 16% to 39% compared with placebo, through changes in hunger, fullness and cravings.

> Mozaffarian et al., Am J Clin Nutr 2025

Less food can mean less muscle.

Four expert societies list muscle and bone loss among the core challenges of GLP-1 care, alongside nutrient gaps from eating less.

> Mozaffarian et al., Am J Clin Nutr 2025

The protein target moves up.

The general adult allowance is 0.8 g/kg a day. Targets of 1.2 to 1.6 g/kg have been proposed during active weight reduction, set for each patient by a clinician.

> Mozaffarian et al., Am J Clin Nutr 2025

Training is the other half.

The advisory says GLP-1 therapy should be prescribed with structured exercise: strength training at least 3 times a week plus 150 minutes of moderate aerobic activity.

> Mozaffarian et al., Am J Clin Nutr 2025

Measure it, don't guess.

The advisory calls for checking strength, function and body composition, and names bioelectrical impedance as a practical point-of-care tool.

> Mozaffarian et al., Am J Clin Nutr 2025

OUR MEDICAL DIRECTOR



Dr. Micah Craig, MD

Board certified in Internal Medicine, Master's in Biochemistry

- Sets our prescribing standards
- Owns the lab schedule and monitoring protocols

How we build nutrition into care.

- Custom meal plan at enrolment, reviewed monthly
- Monthly protein supply included
- Training with the gym's own coaches
- Body composition logged and shared with each patient
- Physician-supervised GLP-1 therapy with labs at each MD consult

THE EVIDENCE, IN ONE LIST

1. GLP-1 therapy cuts how much people eat.
2. Eating less can cost muscle and nutrients.
3. Protein targets rise during weight reduction.
4. Strength training belongs in the prescription.
5. Body composition shows what the scale misses.

Run a gym or a multi-site network?

Book a 15-minute partner call:
vitelizehealth.com/partners/glp1-training

For studios and clinics, ask about our physician-set Peptide Program.

Source: Mozaffarian et al. Nutritional priorities to support GLP-1 therapy for obesity: a joint Advisory from the American College of Lifestyle Medicine, the American Society for Nutrition, the Obesity Medicine Association, and The Obesity Society. Am J Clin Nutr 2025;122:344-367. doi:10.1016/j.ajcnut.2025.04.023. All therapy is prescribed by licensed clinicians only where clinically appropriate. Available in eligible U.S. states.